The second class of market failure in the field of health concerns externalities. Pollution of water or air, and the impact of one infected person upon another, involve negative externalities. Here also, there is market failure and thus a case for state intervention.

'Public health' is defined as the population-scale initiatives that address externalities and provide public goods. Public health mostly involves prevention and not cure: it obtains a healthier population through reduction in sickness. In contrast, most healthcare involves private goods and not public goods.

There is a great deal of confusion around terminology in this field. There is a need for a sharp delineation between four phrases: 'public good', 'market failure', 'public health' and 'healthcare'. The phrase 'public health' is often misunderstood to mean 'healthcare'. Many think that 'public health' is the health of the public. Conversely, it is often incorrectly assumed that 'healthcare' is a 'public good'. The phrase 'public health expenditure' is often applied for government expenditures on healthcare, and it is particularly confusing because it contains the phrase 'public health' which is the antonym of 'healthcare'. The phrase 'government healthcare expenditure' is unambiguous and thus preferable.

Many of the advanced economies of today have been engaged in public health for a very long time. As an example, the UK began work on clean water in 1858 and on clean air in 1952. [2] In many aspects, the public policy initiatives that address externalities and public goods in health are in place, in advanced economies. [3] As a consequence, there is limited interest in public health in the contemporary policy debates of advanced economies.

But in countries such as India, public health is not a solved problem. There is often an inappropriate transfer of concepts and priorities, from the health policy debates of advanced economies, into Indian thinking about health policy. This has given a certain loss of focus upon public health in India.

In 2018, the World Health Organization began a 'Common Goods in Health' (CGH) initiative. [4] The attempt here is to bring back a focus upon the population-scale policy actions, i.e., addressing externalities and providing public goods, in health policy. The phrase 'Common Goods in Health' is unambiguous, and may help reduce the confusion associated with the phrases 'public health', 'public goods', 'healthcare' and 'market failure'.

Healthcare: India has drifted into a largely unregulated private healthcare market. Most healthcare services are purchased from private doctors and hospitals. The private healthcare market suffers from the other two kinds of market failure: asymmetric information and market power.

> *People respond to incentives.*
> *Doctors are people.*
> *Doctors respond to incentives.*

The economic analysis of the behaviour of doctors and hospitals yields great insights. In some ideal world, doctors are altruistic. However, in the real world, the behaviour of doctors is shaped by their self-interest and the objective of revenue maximization.

There is extreme asymmetric information between a doctor and a patient. When a doctor says that a magnetic resonance imaging (MRI) is required, the patient is likely to comply.